

COOK COUNTY CHILDREN'S HOSPITAL

Department of Radiology

1969 W. Ogden Ave., Chicago, IL 60612

Phone: (312) 555-3600 | Fax: (312) 555-3609

CHEST RADIOGRAPH REPORT

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471

DOB: 03/03/2016 **Age:** 9 years

Date of Study: 02/26/2026 **Ordering Provider:** ED Team

Examination: Portable AP Chest Radiograph

CLINICAL INDICATION

Acute respiratory distress, severe asthma exacerbation, intubated. Verify ETT position.

COMPARISON

No prior chest radiographs available in the current system for comparison.

FINDINGS

Endotracheal tube noted with tip approximately 2 cm above the carina — appropriate position. The lungs demonstrate hyperinflation with flattening of the hemidiaphragms bilaterally, consistent with acute air trapping in the clinical setting of asthma exacerbation.

Right upper lobe airspace opacity is noted, measuring approximately 3 cm, with associated perihilar fullness and right hilar lymphadenopathy. The opacity is ill-defined and does not appear to represent simple atelectasis given its morphology and associated hilar prominence.

The left lung is clear. No pneumothorax. No pleural effusion. Cardiac silhouette is normal in size. Mediastinal contours are unremarkable.

IMPRESSION

1. Endotracheal tube in appropriate position.
2. Hyperinflation consistent with acute asthma.
3. Right upper lobe airspace opacity with associated hilar lymphadenopathy. Differential diagnosis includes: atypical pneumonia (Mycoplasma, Chlamydophila), tuberculosis (particularly given the right upper lobe location and hilar lymphadenopathy pattern), fungal infection, or less likely lymphoma. Clinical correlation with patient history is recommended. If the patient has risk factors for tuberculosis or immunosuppression, further evaluation including CT chest and/or infectious workup should be considered. Outpatient follow-up imaging recommended to document resolution.

Interpreted by: Robert Flanagan, MD — Pediatric Radiology

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